

ZS Case Frameworks — A Complete Playbook

A reference to learn the underlying logic once, then apply it to any case. Read it top to bottom the first time. After that, you'll mostly reuse the **Universal Spine** + the one archetype that fits the case in front of you.

Part 0 — How a ZS Case Actually Runs (read this first)

A ZS case is **not** an open MBB case where you drive everything. It's a **guided sequence**:

One opening framework → a chain of small exhibit-based calculations → a final synthesis with recommendation, risks, and next steps.

So your job splits into three skills:

1. Open with structure

(you lead this part — propose a clean framework)

2. Work the data fast and out loud

(they lead this part — feed you exhibits and quant steps)

3. Synthesize

(you lead again — tie it back to the objective)

ZS grades **how you think, not the final number**. Structure, narrating your logic, sanity-checking, and adapting when nudged matter more than landing the exact figure.

The Universal Spine (use on every single case)

1. Clarify & restate the objective.

Repeat the goal back and pin the success metric. Ask 2–3 clarifying questions before structuring. Always nail down:

- What's the metric? (profit / market share / revenue / votes / cost saved)
 - What's the timeframe / constraint?
 - What does "success" look like to the client?
-

2. Structure out loud, then get buy-in.

State your framework as 3–4 MECE buckets.

End with:

"Does it make sense if I start here?"

This shows collaboration (a ZS value) and lets them redirect you early.

3. State a hypothesis.

"My initial hypothesis is X — let's test it."

Gives your analysis direction instead of wandering.

4. Analyze — one insight per exhibit.

For every chart/table:

- do the math
- then say the **insight in one sentence tied to the objective**.

Don't just read numbers aloud — interpret them.

5. Synthesize.

"My recommendation is X, for three reasons: 1, 2, 3."

Lead with the answer, then support it.

6. Risks & next steps.

"The main risk is Y; I'd validate it by doing Z."

Almost every ZS case ends here — don't forget it.

Behaviors that score points

- Think aloud constantly — silence reads as being stuck.
 - Label every number ("this is cost per dose").
 - Sanity-check ("16.5M patients — that feels right for 5% of the US").
 - Ask before assuming; confirm estimates with the interviewer.
 - Adapt instantly when they nudge — they're testing coachability, not catching you out.
 - Stay calm on a number you fumble — recover the logic, the logic is what's graded.
-

Mental-math toolkit (drill these to reflex)

- **Round aggressively:** US population $\approx$ 330M, India $\approx$ 1.4B. Use clean numbers, refine later.

- **Margin** = Price – Cost.
 - **Margin %** = Margin ÷ Price.
 - **Revenue** = Price × Volume.
 - **Profit** = (Price – Cost) × Volume – Fixed costs.
 - **Break-even volume** = Fixed costs ÷ Margin per unit.
 - **Break-even share** = Break-even volume ÷ Market size.
 - **Payback** = Investment ÷ Annual profit.
 - **Market size, two ways:** top-down (population → filter down by %) vs bottom-up (units × price). Cross-check one against the other.
 - **Growth:** rough CAGR — doubling $\approx 70 \div \text{growth-rate-\% years}$ (rule of 70).
-

Archetype 1 — PRICING

You'll recognize it when:

- "help us set a price"
 - "what should it cost"
 - "price a new product"
-

The real question

What price best serves the client's objective (profit, share, or both), given what it costs us, what it's worth to the customer, and what competitors charge?

The framework: triangulate three lenses

Think of it as a **floor, a ceiling, and an anchor**.

Cost-based (the FLOOR)

What does one unit cost to make and deliver?

Price must clear:

- variable cost
- ideally fixed cost + target margin too

Break cost into:

- materials

- labor
- distribution
- overhead

→ Sets the minimum.

Value-based (the CEILING)

What is the product *worth* to the customer — the benefit, outcome, or cost it avoids?

In pharma:

- fewer hospitalizations
- time saved
- better efficacy vs the current standard of care

→ Sets the maximum a rational buyer would pay.

Competition-based (the ANCHOR)

What do alternatives charge, and how good are they relative to us?

- Better than rivals → room for a premium.
- Price-sensitive customers or strong cheap rivals → anchored near the lowest credible price.

→ Sets the realistic market reference.

Where you land between floor and ceiling = the objective

Maximize profit

→ push toward the value ceiling; accept lower volume.

Maximize share fast

→ price at/below competition; accept thinner margin.

Both

(the common pharma goal)

→ match the lowest credible competitor price *where you still earn a strong margin*, and win on differentiation rather than discount.

Pricing-strategy vocabulary

- **Penetration:** low price to grab share fast.
 - **Skimming:** high price early (early adopters), lower over time.
 - **Premium:** sustained high price backed by strong differentiation.
 - **Parity / competitive:** match the market.
 - **Value-based:** price as a fraction of the quantified value delivered.
-

Math you'll likely do

- $\text{Cost per unit} = \text{total cost} \div \text{units}$ (do it per cost component, then sum).
 - $\text{Margin} = \text{Price} - \text{Cost}$; $\text{Margin \%} = \text{Margin} \div \text{Price}$.
 - $\text{Revenue} = \text{Price} \times \text{Volume}$; $\text{Profit} = \text{Margin} \times \text{Volume} - \text{Fixed}$.
 - Elasticity intuition: raise price $\rightarrow$ volume falls; ask whether revenue net rises or falls.
 - Pharma chain: $\text{patients} \times \text{penetration} \times \text{doses/year} \times \text{price} = \text{revenue}$.
-

Pharma lens (weight this heavily for ZS)

- Doses per year
 - Patient pool
 - **Payer reimbursement & market access**
 - Gross-to-net (list price vs net after rebates)
 - Standard of care as the value benchmark
-

Common traps

- Pricing only on cost (ignoring value)
 - Forgetting the volume tradeoff
 - Losing sight of the stated objective
 - Ignoring how competitors will react
-

Strong opening line

"I'd triangulate three things: the cost floor — what a unit costs us; the value ceiling — what the benefit is worth to patients and payers versus current options; and the competitive anchor — what rivals charge and how we compare. Then I'd choose a point between floor and ceiling that fits the client's goal. First — is the goal profit, share, or both?"

Archetype 2 — MARKET ENTRY

You'll recognize it when:

- "should we enter market X"
- "expand into"
- "is this new geography/segment worth it"

The real question

Is this market attractive, can *we* win in it, and is it worth it versus our alternatives?

The framework: four buckets

1. Market attractiveness

- Size
- Growth rate
- Profitability/margins
- Structure (fragmented vs concentrated)
- Regulatory barriers

2. Competition

- Who's already there
- Their share and strengths
- Barriers to entry
- How they'd likely react to us

3. Our right to win (capabilities)

What advantage do we bring?

- differentiation
- cost position
- brand
- distribution

- synergies with our existing business

Without an edge, an attractive market is still a trap.

4. Financials / feasibility

- Investment required
- Break-even (volume and share needed)
- Payback period
- Risk

And:

- does it beat the alternative uses of that capital?
-

The decision rule

Enter only if:

(market is attractive) AND (we can realistically win the share needed to hit the financial target in the timeframe).

The killer test is almost always:

break-even share required vs. the share we could realistically capture.

If you need 10% share in 3 years in a market with entrenched rivals, that's the red flag.

Math (this is exactly the airlines sample case)

- Break-even volume = Fixed costs ÷ (Price – Variable cost).
 - Break-even share = Break-even volume ÷ Market size.
 - Payback = Investment ÷ Annual profit. (NPV intuition if multi-year.)
-

Entry-mode vocabulary

- Organic build
- Acquisition
- Joint venture/partnership
- Licensing

Tradeoffs:

speed vs control vs cost vs risk

Common traps

- Falling in love with a big market while ignoring whether you can win
 - Ignoring competitor response
 - Ignoring time-to-break-even
 - Not comparing the target market against the other options on the table
-

Strong opening line

"I'd answer in four parts: is the market attractive — size, growth, margins; the competitive landscape and barriers; our right to win — the advantage we bring; and the financials — the share we'd need to break even in the client's timeframe and whether that's realistic. The decision really hinges on that last test. Does the client have a target break-even timeframe?"

Archetype 3 — PRODUCT LAUNCH

You'll recognize it when:

- "planning to launch"
 - "go-to-market for a new product"
 - "prepare for launch"
-

The real question

How do we successfully bring this product to market — what must be decided, and what will it achieve?

Launch is broader than entry:

- entry asks *should we go in*
 - launch assumes we're going and asks *how, and how big.*
-

The framework

Market → Product → Go-to-market → Forecast → Risks

1. Market & customer

- Who's the target segment?
 - Size
 - Unmet need
 - Customer/patient journey:
 - awareness
 - purchase
 - repeat
-

2. Product positioning

- Differentiation
 - Value proposition vs existing options
 - Premium or value play
 - Branding
 - **Co-positioning with our existing portfolio so we don't cannibalize our own products**
-

3. Go-to-market — the 4 P's

Product

- features
- formulation
- indication

Price

(plug in the Pricing framework above)

Place

- channels
- distribution
- for pharma: *which physicians and sites of care*

Promotion

- marketing
- salesforce
- awareness

- pharma:
 - HCP engagement
 - patient awareness
 - medical education
-

4. Forecast / sizing

Market size → addressable segment → capture rate → **uptake curve over time**

(adoption ramps; it isn't instant)

→ revenue

Then:

capacity check

Can we actually supply the demand we're forecasting?

5. Launch readiness & risks

- Supply/capacity
 - Regulatory approval timing
 - Payer coverage/access
 - Competitor response
 - Salesforce readiness
-

Pharma launch specifics (ZS's home turf)

- Regulatory approval timing
 - **Market access & reimbursement**
 - **Salesforce sizing & targeting**
 - Patient support programs
 - Medical affairs / KOL engagement
 - Launch sequencing across markets
-

Math

Forecast =

target patients × penetration % × adherence × doses/year × price

ramped over the first few years.

Capacity check:

production capacity ÷ doses per patient = patients we can serve

vs market demand.

Common traps

- Ignoring cannibalization of your own products
 - Assuming instant full uptake
 - Ignoring access/reimbursement
 - Ignoring supply capacity
 - Failing to segment the target rather than treating "the market" as one blob
-

Strong opening line

"I'd structure the launch in five parts: the target market and customer need; the product's positioning and differentiation; the go-to-market plan across product, price, place and promotion; a forecast of uptake and revenue over the first few years; and the launch risks — supply, regulatory and payer access. For pharma I'd weight market access and salesforce targeting most. Is the product already approved and reimbursed?"

Archetype 4 — SEGMENTATION / PRIORITIZATION (resource allocation)

You'll recognize it when:

- "which segment/region/customer/channel should we focus on"
- "where should we allocate budget/salesforce/effort"
- "which to prioritize"

Your friend's voter-bank case is exactly this one.

The real question

Where do we put finite resources to maximize total return?

This is the most important archetype to master because:

it's ZS's actual core business

— segmentation, targeting, and salesforce sizing for pharma clients.

The voter case is the same engine with a political skin.

The governing principle

Maximize total return by allocating resources to the segments with the highest **expected marginal return per unit of resource**.

Score each segment on four levers

1. Size / potential

How big is the prize?

- volume
 - revenue
 - votes
 - patients in the category
-

2. Value

Worth per unit:

- margin
 - lifetime value
 - how *decisive* the segment is
 - e.g., swing district vs a safe seat
-

3. Winnability / responsiveness

How much can your effort actually move them?

This is the lever people forget.

A segment:

- already 90% yours
- or 90% lost

has low **marginal** value no matter how big.

What matters is:

the **gap between current and potential** support

and:

how responsive they are to effort.

4. Cost / effort to serve

Resource needed per unit of return.

The allocation rule

Expected return $\approx$ (Size $\times$ Value $\times$ Responsiveness) – Cost.

Rank segments by this.

Pour resources into the top until diminishing returns set in, then move down the list.

Visual tool

2×2 of Potential vs Winnability

- Attack high–high first
 - Selectively invest in high-potential/low-winnability
 - Deprioritize low–low
-

The pharma original (worth knowing — it's the literal ZS model)

Segment physicians by:

- **potential**
- **responsiveness**

Then:

concentrate salesforce effort on the high-potential, high-responsiveness deciles.

Same four levers.

The voter-bank application

Size

= number of voters in the bank.

Value

= how decisive their votes are (swing vs locked).

Winnability

= persuadability and the current-vs-potential support gap.

Don't spend on voters:

- already locked for you
- already locked against you

Cost

= effort/spend to reach and convert them.

Allocation formula

Allocate to the banks with the best:

$(\text{Size} \times \text{Winnability} \times \text{Decisiveness}) \div \text{Cost}$.

The one sentence that wins this case

"Don't target the biggest bank or your most loyal base — target where your marginal effort changes the most outcomes."

If you say that and then back it with the segment math, you've demonstrated exactly the thinking ZS sells to clients.

Math

For each segment:

- estimate the incremental return
- rank
- allocate greedily (highest ROI first)

under the budget/effort constraint.

2x2s worth knowing

- Potential $\times$ Responsiveness (targeting)
 - Growth $\times$ Share (BCG matrix, for portfolio questions)
 - Effort $\times$ Impact (general prioritization)
-

Common traps

- Targeting by size alone
- Over-investing in your loyal base (no marginal gain)
- Ignoring cost-to-serve
- Treating segments as static
- Not stating a clear allocation rule

Strong opening line

"This is a resource-allocation problem, so I'd score each voter bank on four things: its size, how decisive its votes are, how winnable it is — the gap between current and potential support — and the cost to reach it. Then allocate to the banks with the highest expected vote gain per unit of effort. The key is not to chase the biggest bank or over-invest where we're already safe, but to focus where our effort moves the most votes. Shall I work through the segments with the data given?"

Part 5 — Pharma vocabulary cheat sheet (signals "fit" instantly)

- **HCP** — healthcare professional (the prescribing doctor).
 - **Rx / prescription data** — the core data pharma analytics runs on.
 - **Patient funnel / journey** — diagnosed → treated → on your drug → adherent. Leakage at each stage.
 - **Market access / payer / reimbursement** — whether insurers cover the drug; often make-or-break.
 - **Salesforce sizing & targeting** — how many reps, deployed to which doctors. (ZS's flagship offering.)
 - **Gross-to-net** — list price minus rebates/discounts = what you actually collect.
 - **Indication** — the condition a drug is approved to treat.
 - **Standard of care** — the current default treatment; your value benchmark.
 - **Adherence / compliance** — whether patients keep taking it (drives real-world revenue).
 - **Launch sequencing** — the order you roll out across markets/segments.
-

Part 6 — How to use this document

1.

Learn the Universal Spine cold.

It runs every case; the archetypes just fill in step 2's structure.

2.

For each archetype:

- memorize the bucket names
- memorize the opening line

Not word-for-word, but so the structure is automatic.

3.

Then practice.

Do:

- the four ZS website cases first
 - grade against their reveal answers
 - then quant-heavy MBA casebook cases (Kellogg, INSEAD, Ross, Fuqua)
 - filtered for pricing, entry, launch, and segmentation
 - with a pharma/CPG lean
-

4.

After each case, ask:

- Did I open with structure?
- Did I narrate every number?
- Did I give one insight per exhibit?
- Did I recommend with reasons + risks?

Fix the weakest one next time.

Most real cases are a **blend**

(a launch contains pricing; an entry contains a forecast).

Recognize the *primary* archetype, open with its framework, and pull pieces from the others as the data demands.